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Editorial

Nursing theory and management

'Nursing theory is a conceptualization of some aspect of reality [in this case, managerial reality] that pertains to nursing. The conceptualization is articulated for the purpose of describing, explaining, predicting or prescribing nursing acts' (Meleis 2007, p. 37). Thus, obviously, nursing management theories exist for the 'purpose of describing, explaining, predicting or prescribing nursing acts' (Meleis 2007, p. 37) that facilitate the management of the environment in which registered nurses (RNs) work, whatever that environment may be.

Why do nursing managers need to use nursing theory to guide their work? We propose two specific reasons. Without using nursing theory to guide our work as managers, we believe that we would be nurses who are managers, rather than nurse managers. The distinction offers relevance for work environments eager to foster nursing self-management and professional autonomy. Second, we believe that the lack of a nursing theory framework fosters oppressed group behaviour. The literature has demonstrated that nursing has been an oppressed group under the control of physicians and health-care organizations for decades. Oppressed groups are known to discount their own knowledge in preference for the knowledge of the oppressors. In the professional nursing arena, nurse managers who continue to embrace generic or business management and administrative literature, rather than nursing management/administrative theories, perpetuate an oppressed group approach. Today, we propose that nurses reach beyond the oppression prolonged by organizational bureaucracies that prevent RNs from functioning as licensed health-care providers they are licensed to be. If nurse managers are to foster environments in which RNs can function as independent health-care providers, managers need to embrace nursing theories that relate to management and administration.

As you know, not all managers are leaders and not all leaders are managers. We propose that there are three behaviours that are required if a nurse manager is to be considered a nurse leader. First, a nurse manager must value nurses, the profession, the nursing group or team and the art and science of nursing practice. If a formal leader, such as a nurse manager, does not value the profession, its knowledge, the group or its practice, then it is difficult to conclude the manager is a nursing leader.

In addition, to be an effective nursing leader, an individual needs to provide a vision for the nursing group allowing for review and adoption. If a visionary framework is not used, we believe a true nursing leader is not in place. A nurse manager may be able to demonstrate productivity, ensure patient safety and even generate revenue; however, without a vision that reflects the goals of the nursing profession, the gains will be short-lived and void of true leadership.

Finally, a nurse leader must also assist the nursing group to work toward and achieve the vision that has been presented to, and accepted by, the group. Once the vision has been adopted by a nursing group, the leader then assists the group to work toward and, ultimately, achieve that vision through the procurement of appropriate organizational and structural resources and processes.

Nurse leaders can significantly enhance a nursing group's efforts to achieve the group's vision through the reduction of oppressed group behaviours. By embracing, valuing and incorporating the body of knowledge unique to the practice and art of nursing, nurse leaders will not only model positive approaches that strengthen nursing's position in this era of health-care reform, but also foster the proliferation of new knowledge that will be necessary in the decades to come.

Thus, the editors selected 'nursing theory' as the theme to guide this issue of the *Journal of Nursing Management*. The majority of the articles included in this issue demonstrate the use of nursing theory as the foundation and guide for the research. Further, each reflects the attributes necessary for effective and visionary nursing management, ultimately, quality patient care. The concepts of modelling and role modelling, consistency of nurse care givers, power or outcome attainment, project management, and well-being of aging nurses all shed light on the factors that influence successful nursing care and the role of nurse managers.

Alligood provides us with an illustrated example of how the processes of Modeling and Role Modeling nursing theory can be used by nursing management.

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This strategy can then improve care quality and nurse satisfaction.

Joseph examined the sustainable adoption of whole-person care (WPC) within a health-care organization. In her article, she demonstrates the process of using a nursing theory as an analytical framework to examine the implementation of WPC within a health-care organization and identify suggestions for future implementations.

Mefford explored the influence of the intensity of nursing care and consistency of nursing caregivers on selected health and economic outcomes using Levine's Conservation Model of Nursing as the guiding theoretical framework. She identifies that consistency of nursing caregivers serve as a powerful mediator of the resource utilization outcomes of length of stay, durations of mechanical ventilation, supplemental oxygen therapy and parenteral nutrition.

Although the concept of power or outcome attainment may have a historically negative perception, its influence has significant impact on patient outcomes and patient safety. Gianfermi and Buchholz explored the relationship between job satisfaction and nursing group outcome attainment capability (power capability) in nurse administrators. These researchers found significant correlations between nursing group outcome attainment capability and job satisfaction, leading to the suggestion that improving nursing group outcome attainment capability could lead to increased job satisfaction of nurse administrators and, possibly, increased retention.

Sieloff and Bularzik highlight their work to further validate the Sieloff–King instrument used to measure the outcome attainment capability or power of nursing groups. The ability of nursing groups to attain outcomes, or essentially use their power to influence change, is a critical factor in the success of a heath-care agency.

We are fortunate to be able also include two articles that focus on a qualitative approach to research. This is important as both quantitative and qualitative research strategies are important in the development of nursing knowledge. Through interviews with nurse managers, nurses and public health nurses, Suhonen and Paasivaara were able to identify the main challenges in project management for managers in health care units. This knowledge may serve as the basis for the future development of a theory of project management that may assist all nurse managers as they seek to implement projects within their work setting.

Utriainen, Kyngäs and Nikkilä developed a theoretical model of ageing hospital nurses' well-being at work, interviewing 21 nurses, born between 1948 and 1962. This is critically important work as the nursing shortage continues and health care organizations seek to retain experienced nurses.

Finally, we have included two articles that are based on situational leadership. Although this is not a nursing theory, these articles complement the other articles provided in this issue. Furtado, Batista and Silva used situational leadership to describe nurse managers' leadership behaviours, comparing these behaviours with staff nurses' perceptions of their leaders' leadership, as well as determining whether leadership components affected job satisfaction among staff nurses. Lynch, McCormack and McCance developed a composite model of situational leadership enacted within a person-centered nursing framework in residential care for older people.

We invite you to peruse the included literature and reflect upon the nursing theory content you tried to ignore back in nursing school. At the time, you most likely struggled with the question of how this content could possibly be relevant to your overwhelming need to insert an IV for the first time. Hopefully, the authors have illuminated the liberating role of nursing's unique body of knowledge and expanded your repertoire of nursing management strategies that further the practice as well as the profession.

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Reference

Meleis A.I. (2007) *Theoretical Nursing: Development and Progress*, 4th edn. Lippincott Williams & Wilkins, Philadelphia, PA.

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